Global TB data collection form 2026

A – Annual data collection

1 - IDENTIFICATION

Please enter each name only once. People with accounts to use the WHO global TB data collection system will also be acknowledged in the WHO Global TB Report.

1.1 - Names to cite in the acknowledgements of the WHO Global TB Report

Role	Honorific	Given name(s)	Family name(s)	Email
1.1.1 - National TB control programme manager (NTP) or equivalent	ntp_honorific ☐ Dr ☐ Professor ☐ Other	ntp_given_name	ntp_family_name	ntp_email
1.1.2 - Person responsible for entering data 1	rep1_honorific ☐ Dr ☐ Professor ☐ Other	rep1_given_name	rep1_family_name	rep1_email
1.1.3 - Person responsible for entering data 2	rep2_honorific ☐ Dr ☐ Professor ☐ Other	rep2_given_name	rep2_family_name	rep2_email
1.1.4 - Person responsible for entering data 3	rep3_honorific ☐ Dr ☐ Professor ☐ Other	rep3_given_name	rep3_family_name	rep3_email

1.2 – General remarks

1.2.1 – General remarks	remarks_general

2 - TB notifications by history, site, diagnostic method and by age group and sex

All of section 2 is deactivated for the 29 ECDC countries. Instead, we import their data from files sent by ECDC.

Please report **all** people diagnosed with TB and eligible for TB treatment (including those diagnosed with drug-resistant TB), regardless of whether treatment was started or not. People who died or were lost before treatment start should be notified as they are important to include for surveillance purposes and, from a public health perspective, may have contacts that require tracing and follow up.

2.1 – Pulmonary TB notifications by history and diagnostic method

	New cases	Recurrent (relapse) cases	Cases with unknown previous treatment history	Re-registered (previously treated, excluding recurrent) cases
2.1.1 - Pulmonary TB cases, bacteriologically confirmed (positive by WHO-recommended rapid diagnostics such as Xpert MTB/RIF, Ultra, Truenat MTB, MTB Plus, TB-LAMP or LF-LAM; culture positive; smear positive)	pulm_labconf_new	pulm_labconf_rec	pulm_labconf_unk	pulm_labconf_pulm_rereg
2.1.2 - Pulmonary TB cases, clinically diagnosed (Not bacteriologically confirmed as positive for TB, but diagnosed with active TB by a clinician or another medical practitioner who has decided to give the patient a full course of TB treatment) In future, "clinically diagnosed" cases will be called “bacteriologically unconfirmed”. See page 14 of https://www.who.int/publications/i/item/9789240106932.	pulm_clindx_new	pulm_clindx_rec	pulm_clindx_unk	pulm_clindx_pulm_rereg

2.2 – Extrapulmonary TB notifications by history

	New episodes (new, recurrent, and unknown previous treatment history)	Re-registered (previously treated, excluding recurrent) cases
2.2.1 - Extrapulmonary TB cases, bacteriologically confirmed or clinically diagnosed Cases with both pulmonary and extrapulmonary TB are classified as pulmonary TB cases	ep_newinc	ep_ep_rereg

2.3 - Total new episodes and cases notified

2.3.1 - Total new episodes	c_newinc
2.3.2 - Total cases notified	c_notified

2.4 - Episodes of TB among foreign born individuals

2.4.1 - Among all the TB cases reported, total number of TB cases reported among foreign-born individuals (or among non-citizens if that is the criterion used in your country)	notif_foreign

2.5 - New episodes of TB (pulmonary or extrapulmonary, bacteriologically confirmed or clinically diagnosed, drug-susceptible or drug-resistant) by age group and sex

2.5.1 - For which age groups can you provide notifications disaggregated by age group and sex?	agegroup_option 220 0-4, 5-9, 10-14, 15-19, 20-24, 25-34, 35-44, 45-54, 55-64, 65+ 221 0-4, 5-14, 15-24, 25-34, 35-44, 45-54, 55-64, 65+ 222 0-14, 15-24, 25-34, 35-44, 45-54, 55-64, 65+

2.6 - New episodes of TB by age group and sex

	2.6.1 – New episodes of TB by age group and sex											
	0-4	**5-9**	**10-14**	**15-19**	**20-24**	**25-34**	**35-44**	**45-54**	**55-64**	**65+**	**Age unknown**	**Total**
Female	newrel_f04	newrel_f59	newrel_f1014	newrel_f1519	newrel_f2024	newrel_f2534	newrel_f3544	newrel_f4554	newrel_f5564	newrel_f65	newrel_fu	
Male	newrel_m04	newrel_m59	newrel_m1014	newrel_m1519	newrel_m2024	newrel_m2534	newrel_m3544	newrel_m4554	newrel_m5564	newrel_m65	newrel_mu	
Total												

2.7 - Use of WHO-recommended rapid diagnostic tests

WHO-recommended rapid diagnostics include manual and molecular diagnostic tests of low and moderate complexity to detect TB with and without drug resistance detection (such as urine LF-LAM for persons living with HIV, TB-LAMP, Truenat MTB Plus, Xpert MTB/RIF Ultra, Roche cobas MTB, Abbott RealTime MTB, or BD MAX MTB assays)

2.7.1 - Do you have any data on the number of new episodes (new, recurrent, or unknown previous treatment history) tested using a WHO-recommended rapid diagnostic (for example Xpert MTB/RIF Ultra) as the initial diagnostic test	rdx_data_available 0 No 60 Yes, available from our routine surveillance system 62 Yes, available from our routine surveillance system disaggregated by case type 5 Not applicable (because there were no TB cases)
(If 2.7.1 = yes from routine surveillance system) 2.7.2 - Number of new episodes tested using a WHO-recommended rapid diagnostic as the initial diagnostic test (regardless of test result)	newinc_rdx
(If 2.7.1 = yes from routine surveillance system disaggregated by case type) 2.7.3 - Number of new episodes of pulmonary bacteriologically confirmed cases tested using a WHO-recommended rapid diagnostic as the initial diagnostic test (regardless of test result)	newinc_pulm_labconf_rdx
(If 2.7.1 = yes from routine surveillance system disaggregated by case type) 2.7.4 - Number of new episodes of pulmonary clinically diagnosed cases tested using a WHO-recommended rapid diagnostic as the initial diagnostic test (regardless of test result, noting that by definition a positive result means the case should be classified as bacteriologically confirmed)	newinc_pulm_clindx_rdx

(If 2.7.1 = yes from routine surveillance system disaggregated by case type) 2.7.5 - Number of new episodes of extrapulmonary cases tested using a WHO-recommended rapid diagnostic as the initial diagnostic test (regardless of test result)	newinc_ep_rdx

3 - Diagnosis and enrolment on treatment

Sections 3.1 and 3.2 are deactivated for the 29 ECDC countries. Instead, we import their data from files sent by ECDC.

3.1 - Diagnosis and enrolment on treatment

3.1.1 - Number of people diagnosed with TB who died before starting TB treatment	notified_pre-tx_died
3.1.2 - Number of people diagnosed with TB who were lost to follow-up before starting TB treatment	notified_pre-tx_ltfu
3.1.3 - Number of people diagnosed with TB who started TB treatment	notified_tx_started

3.2 – Number of people who started treatment for rifampicin susceptible TB

3.2.1 - Number of people diagnosed with TB with no evidence of rifampicin resistance (susceptible or tests not done) *This is irrespective of whether or not there is any evidence of isoniazid resistance*	nrr
3.2.2 - Number of people who started treatment using regimens designed to treat rifampicin-susceptible TB	nrr_tx
3.2.3 - Number of people living with HIV who started treatment using regimens designed to treat rifampicin-susceptible TB	nrr_hiv_tx

3.3 – Availability of RR-TB treatment initiation data by duration

3.3.1 - Can you provide data on the number of people who started anti-TB treatment by treatment duration for all RR-TB cases?	tx_flg 1 Yes 0 No

(if 3.3.1 = Yes)

3.4 - Number of people diagnosed with TB who started TB treatment by regimen type

3.4.1 - Number of people who started treatment using short (≤12 months) regimens designed to treat rifampicin-resistant TB (RR-TB/MDR-TB/pre-XDR-TB/XDR-TB)	rr_tx_lt12m
3.4.2 - Number of people who started treatment using long (>12 months and ≤24 months) regimens designed to treat rifampicin-resistant TB (RR-TB/MDR-TB/pre-XDR-TB/XDR-TB)	rr_tx_gt12m_lt24m

(if 3.3.1 = No)

3.5 - Diagnosis and enrolment on treatment of TB patients with laboratory-confirmed rifampicin resistance and no evidence of fluoroquinolone resistance

Calculating indicators related to drug-resistant TB detection and enrolment on treatment requires data on notified TB cases recorded in the basic management unit (BMU) TB register, not from laboratory registers

3.5.1 - Number of people diagnosed with TB with laboratory-confirmed rifampicin resistance and no evidence of fluoroquinolone resistance (susceptible or tests not done) *This should not include pre-XDR-TB and XDR-TB patients.*	conf_rr_nfqr
3.5.2 - Number of patients with laboratory-confirmed rifampicin resistance and no evidence of fluoroquinolone resistance (susceptible or tests not done) started on treatment for MDR/RR-TB *Pulmonary or extrapulmonary. Also include patients diagnosed before the reporting year but started on treatment during the reporting year.* *This should not include pre-XDR-TB and XDR-TB patients treated for pre-XDR-TB or XDR-TB.*	conf_rr_nfqr_tx
3.5.3 - Number of patients without laboratory confirmation of rifampicin resistance started on treatment for MDR/RR-TB *For example, contacts of people with MDR/RR-TB who are started on treatment without laboratory confirmation.* *Pulmonary or extrapulmonary. Also include patients diagnosed before the reporting year but started on treatment during the reporting year.* *This should not include pre-XDR-TB and XDR-TB patients treated for pre-XDR-TB or XDR-TB.*	unconf_rr_nfqr_tx
3.5.4 - Among patients who started treatment for MDR/RR-TB, the number who were aged 0-14 years *Pulmonary or extrapulmonary. Also include patients diagnosed before the reporting year but started on treatment during the reporting year.* *This should not include pre-XDR-TB and XDR-TB patients treated for pre-XDR-TB or XDR-TB.*	rr_nfqr_014_tx

(if 3.3.1 = No)

3.6 - Diagnosis and enrolment on treatment of TB patients with laboratory-confirmed rifampicin resistance and also fluoroquinolone resistance (pre-XDR-TB or XDR-TB)

TB resistant to rifampicin and to any fluoroquinolone is now called pre-XDR-TB. Pre-XDR-TB that is also resistant to at least one of bedaquiline or linezolid is now called XDR-TB.

These new definitions were published by WHO in January 2022. See https://www.who.int/publications/i/item/meeting-report-of-the-who-expert-consultation-on-the-definition-of-extensively-drug-resistant-tuberculosis

3.6.1 - Number with laboratory-confirmed rifampicin resistance and also fluoroquinolone resistance (i.e. pre-XDR-TB or XDR-TB) *Pulmonary or extrapulmonary. Also include patients diagnosed with rifampicin resistance before the reporting period and then with fluoroquinolone resistance during the reporting period.*	conf_rr_fqr
3.6.2 - Number with laboratory-confirmed rifampicin resistance and also fluoroquinolone resistance (i.e. pre-XDR-TB or XDR-TB) who started treatment for pre-XDR-TB or XDR-TB *Pulmonary or extrapulmonary. Also include patients diagnosed before the reporting period but started on treatment during the reporting period.*	conf_rr_fqr_tx

3.7 - Monitoring the use of specific treatment regimens

3.7.1 - Were any TB patients started on the 4-month HPMZ regimen for the treatment of rifampicin-susceptible TB during the reporting year?	nrr_hpmz_used 1 Yes 0 No 3 Don't know
(If 3.7.1 = yes) 3.7.2 - Number of patients started on the 4-month HPMZ regimen	nrr_hpmz_tx
3.7.3 - Were any TB patients started on the 4-month 2HRZ(E)/ 2HR regimen for the treatment of non-severe rifampicin-susceptible TB in people aged 16 years or less during the reporting year?	nrr_2hrze2hr_used 1 Yes 0 No 3 Don't know
(If 3.7.3 = yes) 3.7.4 - Number of patients started on the 4-month 2HRZ(E)/ 2HR regimen in people aged 16 years or less	nrr_2hrze2hr_tx
3.7.5 - Were any TB patients started on a 6-month regimen for the treatment of MDR/RR-TB or pre-XDR-TB during the reporting year? For example, the BPaLM/BPaL regimen or the BDLLfxC regimen. See WHO rapid communication of June 2024 at https://www.who.int/publications/i/item/B09123	dr_6m_used 1 Yes 0 No 3 Don't know
(If 3.7.5 = yes) 3.7.6 - Number of patients started on a 6-month regimen for the treatment of MDR/RR-TB or pre-XDR-TB	dr_6m_tx
3.7.7 - Were any TB patients started on a 9-month regimen for the treatment of MDR/RR-TB during the reporting year? For example, the 9-month or modified 9-month regimens. See WHO rapid communication of June 2024 at https://www.who.int/publications/i/item/B09123	dr_9m_used 1 Yes 0 No 3 Don't know
(If 3.7.7 = yes) 3.7.8 - Number of patients started on a 9-month regimen for the treatment of MDR/RR-TB	dr_9m_tx
3.7.9 - Were any TB patients started on longer regimens of 18 months or more for the treatment of drug-resistant TB during the reporting year? These regimens last 18 months or more and may be individualized and are now recommended only for patients with more extensive resistance (e.g., XDR-TB) or those who are not eligible (e.g., complicated extrapulmonary TB) for or have failed shorter treatment regimens. See WHO rapid communication of June 2024 at https://www.who.int/publications/i/item/B09123	dr_18mplus_used 1 Yes 0 No 3 Don't know
(If 3.7.9 = yes) 3.7.10 - Number of patients started on a longer regimen of 18 months or more for the treatment of drug-resistant TB	dr_18mplus_tx
3.7.11 - Number of patients who started treatment for MDR/RR-TB, pre-XDR-TB or XDR-TB who are also being actively monitored for adverse events This refers to the active and systematic clinical and laboratory assessment of patients on treatment with new anti-TB drugs, novel MDR-TB regimens or XDR-TB regimens to detect, manage and report suspected or confirmed drug toxicities. See the WHO aDSM Implementation Framework	mdr_tx_adsm

4 – Anti-tuberculosis Drug resistance: Surveillance

All of section 4 is deactivated for the 29 ECDC countries. Instead, we import their data from files sent by ECDC.

4.1 - Rifampicin susceptibility testing among people with pulmonary bacteriologically confirmed TB

	New cases	**Previously treated for TB (recurrent and re-registered cases)**	**Cases with unknown previous treatment history**	Total
4.1.1 - Among people with bacteriologically confirmed pulmonary TB, number with documented test results for rifampicin (i.e. positive or negative results only)	r_rlt_new	r_rlt_ret	r_rlt_unk	
4.1.2 - Among people with documented test results for rifampicin, number with resistance to rifampicin (RR-TB)	rr_new	rr_ret	rr_unk	

Section 4.2 is only shown for countries in the high MDR-TB burden list.

4.2 - Rifampicin testing among recurrent (relapse) cases only

4.2.1 - Among people with recurrent (relapse) bacteriologically confirmed pulmonary TB, number with documented test results for rifampicin (i.e. positive or negative results only)	r_rlt_rec
4.2.2 - Among people with recurrent (relapse) bacteriologically confirmed pulmonary TB, number with resistance to rifampicin (i.e. RR-TB).	rr_rec

4.3 - Isoniazid susceptibility testing among people with pulmonary bacteriologically confirmed TB who have also had rifampicin susceptibility testing among people who have never been treated for TB (new cases)

	4.3.1 - Isoniazid susceptibility testing among people with pulmonary bacteriologically confirmed TB who have also had rifampicin susceptibility testing among people who have never been treated for TB (new cases)		
	Resistant to isoniazid	**Susceptible to isoniazid**	**Total**
Resistant to rifampicin	dst_new_rr_hr	dst_new_rr_hs	
Susceptible to rifampicin	dst_new_rs_hr	dst_new_rs_hs	
Total new patients tested for both rifampicin and isoniazid			

4.4 - Isoniazid susceptibility testing among people with pulmonary bacteriologically confirmed TB who have also had rifampicin susceptibility testing among people previously treated (recurrent and re-registered cases)

	4.4.1 - Isoniazid susceptibility testing among people with pulmonary bacteriologically confirmed TB who have also had rifampicin susceptibility testing among people previously treated (recurrent and re-registered cases)		
	Resistant to isoniazid	**Susceptible to isoniazid**	**Total**
Resistant to rifampicin	dst_ret_rr_hr	dst_ret_rr_hs	
Susceptible to rifampicin	dst_ret_rs_hr	dst_ret_rs_hs	
Total previously treated (including recurrent) patients tested for both rifampicin and isoniazid			

4.5 - Fluoroquinolone susceptibility testing among people with pulmonary bacteriologically confirmed rifampicin-resistant TB (RR-TB)

4.5.1 - Among people with RR-TB, number with documented test results (i.e. positive or negative results only) for any fluoroquinolone	rr_dst_rlt_fq
4.5.2 - Among people with RR-TB and with documented test results for fluoroquinolones, number with resistance to any fluoroquinolone (pre-XDR-TB)	rr_fqr

4.6 - Bedaquiline and/or linezolid susceptibility testing among people with pulmonary bacteriologically confirmed rifampicin-resistant TB (RR-TB) and fluoroquinolone-resistant TB (pre-XDR-TB)

	4.6.1 - Bedaquiline and/or linezolid susceptibility test results among people with pulmonary bacteriologically confirmed rifampicin-resistant TB (RR-TB) and fluoroquinolone-resistant TB		
	Resistant to bedaquiline	**Susceptible to bedaquiline**	**Unknown to bedaquiline (i.e. no documented test results)**
Resistant to linezolid	rr_fqr_bdqr_lzdr	rr_fqr_bdqs_lzdr	rr_fqr_bdqu_lzdr
Susceptible to linezolid	rr_fqr_bdqr_lzds	rr_fqr_bdqs_lzds	rr_fqr_bdqu_lzds
Unknown to linezolid (i.e. no documented test results)	rr_fqr_bdqr_lzdu	rr_fqr_bdqs_lzdu	rr_fqr_bdqu_lzdu
Total number with XDR-TB			
Total pre-XDR-TB cases tested			

Lines 4.7.2 to 4.7.4 are only shown for countries in the WHO European region.

4.7 - Testing for resistance to other drugs among people with pulmonary bacteriologically confirmed rifampicin-resistant TB (RR-TB)

	Total number with test results for the drug	Number with resistance to the drug
4.7.1 - Bedaquiline	rr_dst_rlt_bdq	rr_bdqr
4.7.2 - Linezolid	rr_dst_rlt_lzd	rr_lzdr
4.7.3 - Clofazimine	rr_dst_rlt_cfz	rr_cfzr
4.7.4 - Delamanid	rr_dst_rlt_dlm	rr_dlmr

5 - TB/HIV

Questions 5.1.2 and 5.1.3 are deactivated for 22 of the 29 ECDC countries. Instead, we import their data from files sent by ECDC.

5.1 – Testing for HIV

5.1.1 - Total number of new episodes (new, recurrent, or unknown previous treatment history) of TB	c_newinc
5.1.2- Number of people with new episodes of TB who were tested for HIV at the time of TB diagnosis or with known HIV status at the time of TB diagnosis This should include TB patients who were known to be HIV-positive (e.g. documented evidence of enrolment in HIV care such as enrolment in the pre-ART register or in the ART register once started on ART) or with documented negative HIV test conducted at the time of TB diagnosis. If the patient's HIV status is subsequently determined, he or she should be reclassified accordingly.	newrel_hivtest
5.1.3 - Among people with new episodes of TB tested for HIV, the number recorded as HIV positive	newrel_hivpos

5.2 – Antiretroviral therapy

5.2.1 - Among HIV-positive people with new episodes of TB, the number who started or continued on antiretroviral therapy (ART) (Number of people living with HIV with new episodes of TB started on TB treatment during the reporting period who were already on antiretroviral therapy or started on antiretroviral therapy during TB treatment within the reporting year)	newrel_art

Section 5.3 is only shown for countries in the 2026-2030 list of high TB/HIV burden countries and all countries in AMR.

5.3 TB/HIV in people aged 0-14 years

5.3.1 - Do you have a case-based surveillance system that allows you to report on TB/HIV indicators for people aged 0-14 years?	tbhiv_014_flg 1 Yes 0 No
(If 5.3.1 = yes) 5.3.2 - Number of people with new episodes of TB notified aged 0-14 years tested for HIV at the time of TB diagnosis or with known HIV status at the time of TB diagnosis. This should include TB patients who were known to be HIV-positive (e.g. documented evidence of enrolment in HIV care such as enrolment in the pre-ART register or in the ART register once started on ART) or with documented negative HIV test conducted at the time of TB diagnosis. If the patient's HIV status is subsequently determined, he or she should be reclassified accordingly.	newrel_hivtest_014
(If 5.3.1 = yes) 5.3.3 - Among people with new episodes of TB aged 0-14 years, the number recorded as HIV positive	newrel_hivpos_014
(If 5.3.1 = yes) 5.3.4 - Among HIV-positive people with new episodes of TB aged 0-14 years, the number who started or continued on antiretroviral therapy (ART)	newrel_art_014

Global AIDS Monitoring

Global AIDS Monitoring data imported from UNAIDS (https://aidsreportingtool.unaids.org/), as supplied by National AIDS Programme respondents:

People living with HIV who started tuberculosis preventive therapy

	Year TPT initiated	
Total number of people living with HIV newly enrolled on antiretroviral therapy who also started tuberculosis preventive treatment during the same reporting period	2025	hiv_new_tpt
Total number of people living with HIV newly enrolled on antiretroviral therapy (i.e., those registered for antiretroviral therapy during the reporting period)	2025	hiv_new
Total number of people living with HIV currently enrolled on antiretroviral therapy who started tuberculosis preventive treatment during the reporting period	2025	hiv_all_tpt
Total number of people living with HIV currently enrolled on antiretroviral therapy	2025	hiv_all

People living with HIV on antiretroviral therapy who completed a course of TB preventive treatment

	Year TPT initiated	
Number of people on antiretroviral therapy who completed TPT among those who initiated any course of TPT	2024	hiv_all_tpt_completed
Number of people on antiretroviral therapy who initiated any course of TPT	2024	hiv_all_tpt_started

6 – Treatment outcomes

All of section 6 is deactivated for the 29 ECDC countries. Instead, we import their data from files sent by ECDC.

6.1 - Treatment outcomes for people started on regimens designed to treat rifampicin-susceptible TB

We have amended our forms to align with the 2024 guidance on TB surveillance published in May 2024 at https://www.who.int/publications/i/item/9789240075290. In particular, please see Table 4.6.

This now includes all case histories. Ensure that outcomes for "new and recurrent" and "previously treated (excluding recurrent)" are added together.

	Cohort year	**Treatment successful (Cured or treatment completed)**	**Treatment failed**	**Died for any reason after starting TB treatment**	**Lost to follow-up after starting TB treatment**	**Not evaluated**	**Number started treatment (Cohort size)**
6.1.1 - People (irrespective of their HIV status) started on regimens designed to treat rifampicin-susceptible TB	2024	nrr_tx_succ	nrr_tx_fail	nrr_tx_died_after_tx	nrr_tx_lost_after_tx	nrr_tx_neval	nrr_tx_coh
6.1.2 - People living with HIV started on regimens designed to treat rifampicin-susceptible TB	2024	nrr_hiv_tx_succ	nrr_hiv_tx_fail	nrr_hiv_tx_died_after_tx	nrr_hiv_tx_lost_after_tx	nrr_hiv_tx_neval	nrr_hivtx_coh

6.2 – Availability of treatment outcomes for people aged 0-14 years started on regimens designed to treat rifampicin-susceptible TB

	Cohort year	
6.2.1 - Do you have a case-based surveillance system that allows you to report on treatment outcomes for people aged 0-14 years?	2024	outcomes_014_flg 1 Yes 0 No

6.3 - Treatment outcomes for people aged 0-14 years started on regimens designed to treat rifampicin-susceptible TB

This now includes all case histories. Ensure that outcomes for "new and recurrent" and "previously treated (excluding recurrent)" are added together.

	Cohort year	**Treatment successful (Cured or treatment completed)**	**Treatment failed**	**Died for any reason after starting TB treatment**	**Lost to follow-up after starting TB treatment**	**Not evaluated**	**Number started treatment (Cohort size)**
6.3.1 - People (irrespective of their HIV status) started on regimens designed to treat rifampicin-susceptible TB	2024						
(If 6.2.1 = yes) 6.3.2 - People aged 0-14 years started on regimens designed to treat rifampicin-susceptible TB	2024	nrr_tx_014_succ	nrr_tx_014_fail	nrr_tx_014_died_after_tx	nrr_tx_014_lost_after_tx	nrr_tx_014_neval	nrr_tx_014_coh
(If 6.2.1 = yes) People aged 15 years and over started on regimens designed to treat rifampicin-susceptible TB	2024						

6.4 – Availability of treatment outcomes for females (all ages) started on regimens designed to treat rifampicin-susceptible TB

	Cohort year	
6.4.1 - Do you have a case-based surveillance system that allows you to report on treatment outcomes disaggregated by sex?	2024	outcomes_sex_flg 1 Yes 0 No

6.5 - Treatment outcomes for females (all ages) started on regimens designed to treat rifampicin-susceptible TB

This now includes all case histories. Ensure that outcomes for "new and recurrent" and "previously treated (excluding recurrent)" are added together.

	Cohort year	**Treatment successful (Cured or treatment completed)**	**Treatment failed**	**Died for any reason after starting TB treatment**	**Lost to follow-up after starting TB treatment**	**Not evaluated**	**Number started treatment (Cohort size)**
6.5.1 - People (irrespective of their HIV status) started on regimens designed to treat rifampicin-susceptible TB	2024						
(If 6.4.1 = yes) 6.5.2 - Females (all ages) started on regimens designed to treat rifampicin-susceptible TB	2024	nrr_tx_f_succ	nrr_tx_f_fail	nrr_tx_f_died_after_tx	nrr_tx_f_lost_after_tx	nrr_tx_f_neval	nrr_tx_f_coh
(If 6.4.1 = yes) Males (all ages) started on regimens designed to treat rifampicin-susceptible TB	2024						

6.6 - Treatment outcomes for people started on regimens designed to treat rifampicin-resistant TB (RR-TB/MDR-TB/pre-XDR-TB/XDR-TB)

We have amended our forms to align with the 2024 guidance on TB surveillance published in May 2024 at https://www.who.int/publications/i/item/9789240075290. In particular, please see Table 4.6.

*If you respond **Yes** to 6.6.1, please fill in tables 6.7 and 6.8. If you respond **No**, please fill in table 6.9.*

6.6.1 –Do you have data on treatment outcomes for rifampicin-resistant TB by regimen duration for all RR-TB cases (e.g. regimens ≤12 months vs regimens >12 months)?	outcomes_rr_tx_flg 1 Yes 0 No

6.7 - Treatment outcomes for people started on a short (≤12 months) TB treatment regimen designed to treat rifampicin-resistant TB

*Fill in this table only if you responded **Yes** to 6.6.1.*

	Cohort year	**Treatment successful (Cured or treatment completed)**	**Treatment failed**	**Died for any reason after starting TB treatment**	**Lost to follow-up after starting TB treatment**	**Not evaluated**	**Number started treatment (Cohort size)**
6.7.1 - People started on short (≤12 months) regimens designed to treat rifampicin-resistant TB (RR-TB/MDR-TB/pre-XDR-TB/XDR-TB)	2024	rr_tx_lt12m_succ	rr_tx_lt12m_fail	rr_tx_lt12m_died_after_tx	rr_tx_lt12m_lost_after_tx	rr_tx_lt12m_neval	rr_tx_lt12m_coh

6.8 - Treatment outcomes for people started on a longer (>12 months and ≤24 months) TB treatment regimen designed to treat rifampicin-resistant TB

*Fill in this table only if you responded **Yes** to 6.6.1.*

	Cohort year	**Treatment successful (Cured or treatment completed)**	**Treatment failed**	**Died for any reason after starting TB treatment**	**Lost to follow-up after starting TB treatment**	**Not evaluated**	**Total cohort size**
6.8.1 - People started on long (>12 months and ≤24 months) regimens designed to treat rifampicin-resistant TB (RR-TB/MDR-TB/pre-XDR-TB/XDR-TB)	2023	rr_tx_gt12m_lt24m_succ	rr_tx_gt12m_lt24m_fail	rr_tx_gt12m_lt24m_died_after_tx	rr_tx_gt12m_lt24m_lost_after_tx	rr_tx_gt12m_lt24m_neval	rr_tx_gt12m_lt24m_coh

6.9 - Treatment outcomes for patients started on treatment for drug-resistant TB

*Fill in this table only if you responded **No** to 6.6.1.*

	Cohort year	**Treatment successful (Cured or treatment completed)**	**Treatment failed**	**Died for any reason after starting TB treatment**	**Lost to follow-up after starting TB treatment**	**Not evaluated**	**Total cohort size**
6.9.1 - Patients with laboratory-confirmed RR-TB/MDR-TB treated for MDR-TB and not treated for pre-XDR or XDR-TB	2023	mdr_succ	mdr_fail	mdr_died_after_tx	mdr_lost_after_tx	mdr_neval	mdr_coh
6.9.2 - Patients with laboratory-confirmed pre-XDR-TB/XDR-TB treated for pre-XDR or XDR-TB *Do not include these cases in 6.9.1. If you cannot report outcomes for pre-XDR-TB/XDR-TB cases separately from RR-/MDR-TB cases, include all cases in 6.9.1 and add a note in the General Remarks.*	2023	xdr_succ	xdr_fail	xdr_died_after_tx	xdr_lost_after_tx	xdr_neval	xdr_coh

7 - TB deaths recorded in the civil registration and vital statistics system

7.1 - TB deaths recorded in the civil registration and vital statistics system

7.1.1 - Do you have data on deaths recorded in your national civil registration and vital statistics system?	vr_data_available 1 Yes 0 No
(If 7.1.1 = yes) 7.1.2 - Total number of deaths registered by the national civil registration and vital statistics system, including deaths with ICD-10 codes in R00-R99	total_deaths_vr
(If 7.1.1 = yes) 7.1.3 - Number of deaths registered by the national civil registration and vital statistics system with ICD-10 codes in R00-R99	r00_r99_deaths_vr
(If 7.1.1 = yes) 7.1.4 - Number of deaths registered by the national civil registration and vital statistics system with ICD-10 codes for TB, including deaths from TB sequalae	tbdeaths_vr

8.1 - Screening, contact investigation and TB preventive treatment

8.1 - Screening

8.1.1 - Do you have any data on the number of people with TB who were reported through active TB screening efforts as opposed to passive case detection? Examples of active TB screening include screening in contacts, people living with HIV, prisons, mining communities, facility-based screening of people with diabetes and other groups and other active case-finding	tb_from_screen_available 1 Yes 65 No, and there are no plans to collect these data in future 66 No, but we plan to start collecting and reporting these data in future 5 Not applicable (because there were no TB cases)
(If 8.1.1 = yes) 8.1.2 - Among people with new episodes (new, recurrent, or unknown previous treatment history) of TB, the number who were reported through active TB screening efforts	newinc_from_screen

8.2 - Contact investigation

8.2.1 - Do you have any data on the number of household contacts of people with a new episode (new, recurrent, or unknown previous treatment history) of bacteriologically confirmed pulmonary TB who were evaluated for TB?	screen_data_available 1 Yes 65 No, and there are no plans to collect these data in future 66 No, but we plan to start collecting and reporting these data in future 5 Not applicable (because there were no TB cases)
(If 8.2.1 = yes) 8.2.2 - Number of household contacts of people with a new episode of bacteriologically confirmed pulmonary TB	newinc_con
(If 8.2.1 = yes) 8.2.3 - Number of household contacts of people with a new episode of bacteriologically confirmed pulmonary TB who were evaluated for TB disease and TB infection	newinc_con_screen
(If 8.2.1 = yes) 8.2.4 - Among the household contacts evaluated for TB disease and TB infection, the number who were diagnosed with TB disease	newinc_con_tb

8.3 - TB preventive treatment initiation

8.3.1 - Do you have any data on the number of household contacts of people with TB started on TB preventive treatment?	prevtx_data_available 1 Yes 65 No, and there are no plans to collect these data in future 66 No, but we plan to start collecting and reporting these data in future 5 Not applicable (because there were no TB cases)
(If 8.3.1 = yes) 8.3.2 - Number of household contacts of people with a new episode of bacteriologically confirmed pulmonary TB who were started on TB preventive treatment	newinc_con_prevtx

(If 8.3.1 = yes) 8.3.3 - Number of people aged under 5 years started on TB preventive treatment who are household contacts of people with a new episode of bacteriologically confirmed TB in the reporting year	newinc_con04_prevtx

8.4 - Completion of TB preventive treatment

	Cohort year	
8.4.1 - Do you have any data on the number of household contacts of people with TB who were started on TB preventive treatment and who completed the course of treatment?	2024	prevtx_cmplt_data_available 1 Yes 65 No, and there are no plans to collect these data in future 66 No, but we plan to start collecting and reporting these data in future 5 Not applicable (because there were no TB cases)
(If 8.4.1 = yes) 8.4.2 - Among household contacts who had started TB preventive treatment in the previous reporting year, number who completed the course of TB preventive treatment	2024	newinc_con_prevtx_cmplt

8.5 - Shorter TB preventive treatment regimens

8.5.1 - Were any shorter TB preventive treatment regimes containing rifampicin or rifapentine used at least once?	tpt_short_regimens_used 1 Yes 67 No, and there are no plans to do so in future 68 No, but we plan to do so in future 3 Don't know
(If 8.5.1 = yes) 8.5.2 - Was 1-month daily rifapentine and isoniazid (1HP) used at least once?	tpt_1hp 1 Yes 0 No 3 Don't know
(If 8.5.1 = yes) 8.5.3 - Was 3-month weekly rifapentine and isoniazid (3HP) used at least once?	tpt_3hp 1 Yes 0 No 3 Don't know
(If 8.5.1 = yes) 8.5.4 - Was 3-month daily rifampicin and isoniazid (3HR) used at least once?	tpt_3hr 1 Yes 0 No 3 Don't know
(If 8.5.1 = yes) 8.5.5 - Was 4-month daily rifampicin (4R) used at least once?	tpt_4r 1 Yes 0 No 3 Don't know
(If 8.5.1 = yes) 8.5.6 - Total number of people started on shorter TB preventive treatment regimens containing rifapentine (1HP or 3HP) or rifampicin (3HR or 4R) (please also include data from national HIV/AIDS programme)	prevtx_short_rifamycin

(If 8.5.1 = yes and (8.5.2 = yes or 8.5.3 = yes)) 8.5.7 - Total number of people started on shorter TB preventive treatment regimens containing rifapentine (1HP or 3HP) (please also include data from national HIV/AIDS programme)	tpt_rifapentine_tx
8.5.8 - Was TB preventive treatment with 6-month daily levofloxacin (6Lfx) used at least once in contacts exposed to MDR/RR-TB?	tpt_6lfx 1 Yes 0 No 3 Don't know

9 - Multisectoral engagement: Detection of TB in prisons

9.1 - Multisectoral engagement: Detection of TB in prisons

9.1.1 - Are data available on the number of people with TB who are prisoners?	ident_pris 1 Yes 0 No
(If 9.1.1 = yes) 9.1.2 - Number of people with a new episode (new, recurrent, or unknown previous treatment history) of TB registered in prisons	newrel_prisoners

10 - TB infection control

Only shown to a subset of countries (criteria: incidence >= 20/100k population or >=10 notified cases and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

10.1 - TB infection control

Data on TB in health workers is typically available from occupational health programmes.
If infection control is effective, the annual TB incidence rates in health workers (relative to the total number of health workers) should not on average exceed annual TB incidence rates in the general population of the same age and sex groups.

10.1.1 - Do you have any data on the occurrence of TB in all workers at health care facilities, not just those working in the TB programme, for example from an occupational health programme?	tb_in_hcw_data_available 1 Yes 0 No 3 Don't know
(If 10.1.1= yes) 10.1.2 - How many workers at health care facilities, including those working outside the TB programme and including non-medical staff such as managers, receptionists or drivers, were working in the country in the public	hcw_tot
(If 10.1.1= yes) 10.1.3 - Number of health-care workers who had TB	hcw_tb_notified *(Name changed during migration from hcw_tb_infected)*

11 – Laboratory diagnostic services

Only shown to a subset of countries (criteria: incidence >= 20/100k population or >=10 notified cases and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

See Framework of indicators and targets for laboratory strengthening under the End TB Strategy

11.1 - Tests of TB infection used in the public or private sector before starting TB preventive treatment for any population at risk

11.1.1 - Interferon Gamma Release Assays (IGRA)	igra_used 1 Yes 0 No 3 Don't know
11.1.2 - Tuberculin Skin Tests (TST)	tst_used 1 Yes 0 No 3 Don't know
11.1.3 - Antigen-based Skin Tests (TBST)	tbst_used 1 Yes 0 No 3 Don't know

11.2 - Molecular WHO-recommended rapid diagnostic testing

11.2.1 - Total number of diagnostic tests performed using molecular WHO-recommended rapid diagnostics These include nucleic acid amplification tests of low and moderate complexity with and without drug resistance detection (TB-LAMP, Truenat MTB Plus and Rif Dx, Xpert MTB/RIF Ultra, Roche cobas MTB, Abbott RealTime MTB, and BD MAX MTB assays)	m_wrd_tests_performed
11.2.2 - Number of positive results among the diagnostic tests performed using molecular WHO-recommended rapid diagnostics	m_wrd_tests_positive

11.3 - Sites performing TB diagnostic testing at the end of reporting year

11.3.1 - Total number of sites providing laboratory diagnostic testing for TB at the end of reporting year Include all sites contributing to the diagnosis of TB, including laboratories within or outside the public health sector.	dx_test_sites

11.4 - Number of sites providing these services at the end of the reporting year

11.4.1 - Smear microscopy (including fluorescent)	smear
11.4.2 - Culture	culture
11.4.3 - Molecular WHO-recommended rapid diagnostics for detection of MTB These include nucleic acid amplification tests of low and moderate complexity with and without drug resistance detection (TB-LAMP, Truenat MTB Plus and Rif Dx, Xpert MTB/RIF Ultra, Roche cobas MTB, Abbott RealTime MTB, and BD MAX MTB assays)	m_wrd
11.4.4 - Molecular tests for detection of isoniazid resistance (Low or moderate complexity automated nucleic acid amplification tests or first-line line probe assays)	m_inh
11.4.5 - Molecular tests for detection of fluoroquinolone resistance (Low complexity automated nucleic acid amplification tests or second-line line probe assays)	m_fq
11.4.6 - Phenotypic drug susceptibility testing for moxifloxacin and/or levofloxacin	dst_moxlev
11.4.7 - Phenotypic drug susceptibility testing for bedaquiline	dst_bdq
11.4.8 - Phenotypic drug susceptibility testing for linezolid	dst_lzd
11.4.9 - Phenotypic drug susceptibility testing for pretomanid	dst_ptd

12 - Universal access to rapid tuberculosis diagnostics

Only shown to a subset of countries (criteria: high TB and high MDR-TB burden countries plus other high priority countries and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

Universal access to rapid tuberculosis diagnostics

The data in this section are for the 12 benchmarks that form the WHO standard: universal access to rapid TB diagnostics (see https://www.who.int/publications/i/item/9789240071315).

You can use these data to monitor your progress in expanding access to WHO-recommended rapid diagnostics, for example using the dashboard at https://www.who.int/teams/global-tuberculosis-programme/data#profiles, specifically https://worldhealthorg.shinyapps.io/uad_benchmarks/.

You can also use the identified successes and gaps in service delivery to update 2025 and 2026 strategic plans and funding proposals, enabling funders and partners to better align their work with your priorities.

WHO-recommended rapid diagnostic tests include manual and molecular diagnostic tests of low and moderate complexity to detect TB with and without drug resistance detection (such as urine LF-LAM for persons living with HIV, TB-LAMP, Truenat MTB Plus, Xpert MTB/RIF Ultra, Roche cobas MTB, Abbott RealTime MTB, or BD MAX MTB assays)

Step 1: Identifying presumptive TB

12.1 - Benchmark 1: All household contacts, all people living with HIV, and other locally relevant high-risk groups are screened for TB

12.1.1 - Are data available on the number of people living with HIV screened for TB?	plhiv_all_screen_data_available 1 Yes 0 No
(If 12.1.1 = yes) 12.1.2 - Total number of people living with HIV	plhiv_all
(If 12.1.1 = yes) 12.1.3 - Number of people living with HIV who were screened for TB	plhiv_all_screen
12.1.4 - Are data available on the number of people newly diagnosed with HIV and screened for TB?	plhiv_new_screen_data_available 1 Yes 0 No
(If 12.1.4 = yes) 12.1.5 - Number of people newly diagnosed with HIV	plhiv_new
(If 12.1.4 = yes) 12.1.6 - Number of people newly diagnosed with HIV who were screened for TB	plhiv_new_screen
12.1.7 - Are data available on the number of prisoners screened for TB?	prisoners_screen_data_available 1 Yes 0 No
(If 12.1.7 = yes) 12.1.8 - Number of prisoners You can check numbers reported at https://www.prisonstudies.org	prisoners
(If 12.1.7 = yes)	prisoners_screen

12.1.9 - Number of prisoners who were screened for TB	
12.1.10 - Are data available on the number of miners exposed to silica dust screened for TB?	miners_screen_data_available 1 Yes 0 No
(If 12.1.10 = yes) 12.1.11 - Number of miners exposed to silica dust	miners
(If 12.1.10 = yes) 12.1.12 - Number of miners who were screened for TB	miners_screen

12.2 - Benchmark 2: In all districts, chest X-ray is used regularly for TB screening

A district is an officially demarcated area known as a "basic management unit" or "county" in some settings. A basic management unit (BMU) is defined in terms of management, supervision and monitoring responsibility. A BMU for the TB programme may have several treatment facilities, one or more laboratories and one or more hospitals. The defining aspect is the presence of a manager or coordinator who oversees TB control activities for the unit and who maintains a master register of all TB patients being treated. Typically, the units correspond to the government's second subnational administrative division.

12.2.1 - What is the name used in your country for the administrative level known as a "district", a "basic management unit" or a "county" in the definition above?	district_description
12.2.2 - Total number of districts	district
12.2.3 - Number of districts in which chest X-ray was used regularly (with or without CAD) for TB screening (chest X-ray screening every week of the year or at least in quarterly active case-finding campaigns)	district_cxr

Step 2: Accessing testing

12.3 - Benchmark 3: In all facilities in all districts, the TB diagnostic algorithm requires use of a WHO-recommended rapid diagnostic test as the initial diagnostic test for all patients with presumptive TB

12.3.1 - Number of districts in which all facilities have a TB diagnostic algorithm that requires a WHO-recommended rapid diagnostic test to be used as the initial diagnostic test for all individuals with presumptive TB Testing all individuals with presumptive TB also includes testing in children, people living with HIV (combined with lateral flow urine lipoarabinomannan assay) and people with extrapulmonary TB	district_wrd

12.4 - Benchmark 4: All primary health-care facilities have access to WHO-recommended rapid diagnostic tests (on site or through sample referral)

12.4.1 - Total number of primary health-care facilities	phcf
12.4.2 - Number of primary health-care facilities with access to WHO-recommended rapid diagnostic tests (either on site or through a sample referral system)	phcf_wrd

12.5 - Benchmark 5: All individuals with TB have access to a WHO-recommended rapid diagnostic test as the initial diagnostic test

Data will be taken from section 2

12.6 - Benchmark 6: WHO-recommended rapid diagnostic testing capacity meets expected needs, including surge capacity, according to the latest data

Note that a value above 100% may not mean that all people have access, it may mean that the capacity is not optimally divided over the country or that an overcapacity may be needed to provided access also in remote areas.

12.6.1 - Total number of diagnostic tests performed using molecular WHO-recommended rapid diagnostics	m_wrd_tests_performed
12.6.2 - Number of people identified with presumptive TB For guidance on measuring the number of people identified with presumptive TB please see page 13 of The WHO standard: Universal access to rapid tuberculosis diagnostics	presumptive
12.6.3 - Number of WHO-recommended rapid diagnostics tests that can be performed with the existing instruments	wrd_test_capacity

Step 3: Being tested

12.7 - Benchmark 7: All functional instruments have an error rate of 5% or less

12.7.1 - Number of sites providing molecular WHO-recommended rapid diagnostics testing for TB	m_wrd
12.7.2 - Number of sites providing molecular WHO-recommended rapid diagnostics testing for TB with annual error rates of 5% or less	m_wrd_error_rate_lte_5pct

12.8 - Benchmark 8: All individuals with presumptive TB are tested with a WHO-recommended rapid diagnostic test

12.8.1 - Number of people identified with presumptive TB who were tested with a WHO-recommended rapid diagnostic test	presumptive_wrd

12.9 - Benchmark 9: All patients with bacteriologically confirmed TB undergo universal drug susceptibility testing

Data will be taken from section 4

Step 4: Receiving a diagnosis

12.10 - Benchmark 10: An initial WHO-recommended rapid diagnostic test result is available to inform a diagnosis of pulmonary TB

Data will be taken from section 2

12.11 - Benchmark 11: All districts monitor the test positivity rate to optimize the impact of screening and testing strategies

12.11.1 - Number of districts that monitored test positivity rate	district_monitor_pos_rate

12.12 - Benchmark 12: All TB testing laboratories achieve a turn-around time of within 48 hours for at least 80% of samples received for WHO-recommended rapid diagnostic testing

12.12.1 - Number of sites providing molecular WHO-recommended rapid diagnostics testing for TB	m_wrd
12.12.2 - Number of laboratories that achieved a turn-around time within 48 hours for at least 80% of samples received for WHO-recommended rapid diagnostic testing	m_wrd_tat_lt_48h

13 - Multisectoral engagement: Public-Private Mix (PPM)

Only shown to a subset of countries (criteria: known to have public-private mix activities and can report data on those activities and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

13.1 - Multisectoral engagement: Public-Private Mix (PPM)

13.1.1 - Number of people with TB notified by private non-NTP providers Include all contributions from private individual and institutional providers, corporate/business sector providers, mission hospitals, and other clinics/hospitals managed by non-governmental organizations and faith-based organizations	priv_new_dx
13.1.2 - Among the people with TB notified by private non-NTP providers, the number tested using a WHO-recommended rapid diagnostic as the initial diagnostic test (regardless of test result) Pulmonary or extrapulmonary, bacteriologically confirmed or clinically diagnosed, drug-susceptible or drug-resistant	priv_wrd
13.1.3 - Number of people with TB notified by public non-NTP providers Include all contributions from public hospitals, public medical colleges, prisons/detention centres, military facilities, railways, public health insurance organizations etc	pub_new_dx
13.1.4 - Among the people with TB notified by public non-NTP providers, the number tested using a WHO-recommended rapid diagnostic as the initial diagnostic test (regardless of test result) Pulmonary or extrapulmonary, bacteriologically confirmed or clinically diagnosed, drug-susceptible or drug-resistant	pub_wrd

14 - Multisectoral engagement: Community Engagement

Only shown to a subset of countries (criteria: known to have community engagement activities and can report data on those activities and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

14.1 - Multisectoral engagement: Community Engagement

14.1.1 - How many TB basic management units (BMUs) were there? A basic management unit (BMU) is defined in terms of management, supervision and monitoring responsibility. A BMU for the TB programme may have several treatment facilities, one or more laboratories and one or more hospitals. The defining aspect is the presence of a manager or coordinator who oversees TB control activities for the unit and who maintains a master register of all TB patients being treated. Typically, the units correspond to the government's second subnational administrative division, which might be called, for example, a "district" or "county".	bmu
14.1.2 - How many BMUs were implementing community-based referrals or any form of community treatment adherence support?	bmu_community_impl

14.2 - Referrals by community health workers / community volunteers

14.2.1 - Do you have data on community-based referrals?	community_refdata_available 1 Yes 0 No
(If 14.2.1 = yes) 14.2.2 - Number of BMUs with data on referrals by community health workers / community volunteers	bmu_ref_data
(If 14.2.1 = yes) 14.2.3 - Total number of people with new episodes (new, recurrent, and unknown previous treatment history) of TB notified in the Basic Management Units with data on referrals by community health workers	notified_ref
(If 14.2.1 = yes) 14.2.4 - Number of people with new episodes of TB referred by community health workers / community volunteers in the Basic Management Units with data on referrals by community health workers Include contributions from all community health workers / community volunteers including those supervised by the government, non-governmental organizations, community-based organizations, faith-based organizations and patient-based organizations. Community health workers are people with some formal education who are given training to contribute to community-based health services, and their time is often compensated by incentives in kind or in cash. Community volunteers are community members who have been systematically sensitized about TB prevention and care, either through a short, specific training scheme or through repeated, regular contact sessions with professional health workers.	notified_ref_community

14.3 - Treatment adherence support from community health workers / community volunteers for people who started TB treatment

	Period	
14.3.1 - Are data available on any form of community treatment adherence support?	2024	community_outdata_available 1 Yes 0 No
(If 14.3.1 = yes) 14.3.2 - Number of BMUs with data on community treatment adherence support	2024	bmu_rxsupport_data
(If 14.3.1 = yes) 14.3.3 - Total number of people who started TB treatment in the Basic Management Units with data on community treatment adherence support	2024	bmu_rxsupport_data_coh
(If 14.3.1 = yes) 14.3.4 - Total number of people who started TB treatment and who received any form of treatment adherence support from community health workers / community volunteers. Include contributions from all community health workers / community volunteers including those supervised by the government, non-governmental organizations, community-based organizations, faith-based organizations and patient-based organizations. Community health workers are people with some formal education who are given training to contribute to community-based health services, and their time is often compensated by incentives in kind or in cash. Community volunteers are community members who have been systematically sensitized about TB prevention and care, either through a short, specific training scheme or through repeated, regular contact sessions with professional health workers.	2024	rxsupport_community_coh
(If 14.3.1 = yes) 14.3.5 - Number of people with TB who were cured or who completed treatment among the people who received any form of treatment adherence support from community health workers / community volunteers	2024	rxsupport_community_succ

14.4 - Community representation in national decision making

Please indicate if representatives of communities affected by TB or civil society had a formal role in the following:

14.4.1 - Development of the national strategic plan	community_nsp 1 Yes 0 No 2 Not applicable 3 Don't know
14.4.2 - Preparation and conduct of the TB programme review	community_prog_review 1 Yes 0 No 2 Not applicable 3 Don't know
14.4.3 - Development of the national annual TB report	community_annual_report 1 Yes 0 No 2 Not applicable 3 Don't know
14.4.4 - Development or update of national TB guidelines or manuals	community_manuals 1 Yes 0 No 2 Not applicable 3 Don't know

14.5 - Level of committed funding for community engagement in the TB response at national level

	Period	
14.5.1 - Do you have data on the funding committed to community engagement activities in the national TB budget?	2026	community_budget_available 1 Yes 0 No
(If 14.5.1 = yes) 14.5.2 - Expected funding for community engagement activities, in US Dollars This amount should already be included as a component of "Drug-susceptible TB programme costs" in the budget section of the form. It should not include the cost of using community health care workers in TB care delivery	2026	cf_community

15 – Budget and expenditure

Only shown to a subset of countries (criteria: high income countries with incidence < 20/100k population or < 10 notified cases and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

15.1 - Expenditure

15.1.1 - Total expenditure on TB prevention, diagnostic and treatment services	exp_tot

15.2 - Budget

	Fiscal year	
15.2.1 - Total budget required for TB prevention, diagnostic and treatment services	2026	budget_tot

16 - Budget

Only shown to a subset of countries (criteria: low and middle--income countries with incidence >= 20/100k population or >= 10 notified cases and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

16.1 - Budget

Please report all financial data in US Dollars. Please leave data items empty if their values are not known. Enter 0 only if the true value is zero.

Budget line item	Fiscal Year	**Budget required**	**Expected funding**	**Gap**
16.1.1 - Laboratory infrastructure, equipment and supplies Building, maintaining, and renovating TB laboratories, laboratory equipment purchase and maintenance, consumables for all tests (including TB screening for people living with HIV/AIDS and diagnosis of latent TB infection), quality assurance, retooling and the transportation of specimens.	2026	budget_lab	cf_lab	
16.1.2 - National TB Programme staff (central unit staff and subnational TB staff) Salaries and incentives of those working only on TB activities at central and peripheral levels (for example provincial TB coordinators, district TB coordinators, etc.). Do not include primary health care personnel working on other diseases in addition to TB.	2026	budget_staff	cf_staff	
16.1.3 - Drug-susceptible TB: drugs Drugs for patients being treated for drug-susceptible TB. Include children, re-treatment cases and buffer stock.	2026	budget_fld	cf_fld	
16.1.4 - Drug-susceptible TB: programme costs The management and supervision of the TB control programme, training, policy development, meetings, visits for supervision, purchase of office equipment/vehicles, construction of buildings for use by programme staff, routine surveillance, advocacy and communication, public-private mix activities, community engagement, active case-finding, infection control, management of TB drug procurement and distribution, and programme activities linked to contact investigation for TB preventive treatment	2026	budget_prog	cf_prog	
16.1.5 - Drug-resistant TB: drugs Drugs to treat drug-resistant TB (RR-TB, MDR-TB, pre-XDR-TB or XDR-TB). Include drugs to deal with adverse event	2026	budget_sld	cf_sld	
16.1.6 - Drug-resistant TB: programme costs Management of drug-resistant TB services, excluding drugs. Examples are renovation of MDR-TB wards, support for the Green Light Committee, conducting an MDR situation assessment, default and contact tracing, palliative care.	2026	budget_mdrmgt	cf_mdrmgt	

16.1.7 - TB preventive treatment: drugs Drugs for TB preventive treatment, as per latest WHO guidance (6H, 9H, 4R, 3HR and Levofloxacin)	2026	budget_tpt	cf_tpt	
16.1.8 - Collaborative TB/HIV activities Collaboration between TB and HIV programmes aimed at reducing the impact of HIV-related TB. Activities include TB/HIV coordinating bodies, joint TB/HIV training and planning, HIV testing for TB patients, HIV surveillance among TB patients, co-trimoxazole preventive therapy (CPT), joint TB/HIV education/ communication, and antiretroviral treatment for TB patients. TB screening for people living with HIV/AIDS is included under (Lab infrastructure, equipment, and supplies).	2026	budget_tbhiv	cf_tbhiv	
16.1.9 - Patient support Cash transfers, food packages, transportation vouchers, educational and emotional support to patient or other in-kind benefits given to TB patients, mobile phone (airtime or device for V.O.T), medications monitors (digital box, 99 DOTS)	2026	budget_patsup	cf_patsup	
16.1.10 - Operational research and surveys Periodic surveys (prevalence, drug resistance, patient catastrophic cost); routine surveillance (epidemiology review, inventory studies, pharmacovigilance, systematic assessment of the surveillance system); operational research.	2026	budget_orsrvy	cf_orsrvy	
16.1.11 - All other budget lines Please explain these amounts using the comment icon next to the data entry boxes.	2026	budget_oth	cf_oth	
16.1.12 - Total Budget required				
16.1.13 - Total Expected funding				
16.1.14 - Total GAP				

16.2 - Total expected funding

	Fiscal Year	
16.2.1 - Domestic (including loans)	2026	cf_tot_domestic
16.2.2 - Global Fund	2026	cf_tot_gf
16.2.3 - US Government	2026	cf_tot_usaid
16.2.4 - Other sources	2026	cf_tot_grnt
16.2.5 - Total expected funding		

17 - Expenditure

Only shown to a subset of countries (criteria: low and middle--income countries with incidence >= 20/100k population or >= 10 notified cases and in agreement with regional offices). The country list was inherited from the 2025 data collection form with no changes.

17.1 - Expenditure

Please report all financial data in US Dollars. Please leave data items empty if their values are not known. Enter 0 only if the true value is zero.

	Fiscal year	Actual expenditure	Received Funding
17.1.1 - Laboratory infrastructure, equipment and supplies	2025	exp_lab	rcvd_lab
17.1.2 - National TB Programme staff (central unit staff and subnational TB staff)	2025	exp_staff	rcvd_staff
17.1.3 - Drug-susceptible TB: drugs Drugs for patients being treated for drug-susceptible TB. Include children, re-treatment cases and buffer stock.	2025	exp_fld	rcvd_fld
17.1.4 - Drug-susceptible TB: programme costs	2025	exp_prog	rcvd_prog
17.1.5 - Drug-resistant TB: drugs	2025	exp_sld	rcvd_sld
17.1.6 - Drug-resistant TB: programme costs	2025	exp_mdrmgt	rcvd_mdrmgt
17.1.7 - TB preventive treatment: drugs	2025	exp_tpt	rcvd_tpt
17.1.8 - Collaborative TB/HIV activities	2025	exp_tbhiv	rcvd_tbhiv
17.1.9 - Patient support	2025	exp_patsup	rcvd_patsup
17.1.10 - Operational research and surveys	2025	exp_orsrvy	rcvd_orsrvy
17.1.11 - All other budget lines for TB	2025	exp_oth	rcvd_oth
17.1.12 - Total Actual expenditure			
17.1.13 - Total Received Funding			

17.2 - Total received funding

	Fiscal year	
17.2.1 - Domestic (including loans)	2025	rcvd_tot_domestic
17.2.2 - Global Fund	2025	rcvd_tot_gf
17.2.3 - US Government	2025	rcvd_tot_usaid
17.2.4 - Other sources	2025	rcvd_tot_grnt
17.2.5 - Total received funding	2025	

18 - Utilization of health services

18.1 - Utilization of health services

	Patients starting first-line TB treatment	Patients starting MDR-TB / pre-XDR-TB / XDR-TB treatment
18.1.1 - Typical number of visits to a health facility after diagnosis The average number of visits per patient to any health facility during TB treatment, for example for observed treatment (DOT), collection of drugs, smear monitoring, etc. after the patient has been diagnosed with TB, in view of your treatment guidelines. For example, if a TB patient on first-line treatment receives directly observed treatment daily in the intensive phase at clinics and, in the continuation phase 4 visits are required (one per month for collection of drugs), the total would be 60+4=64.	hcfvisit_dstb	hcfvisit_mdr
18.1.2 - Estimated percentage of cases that are hospitalized (%) If the actual percentage of hospitalisations is available from the basic management unit register, please report. If not, please report the approximate percentage of patients hospitalized for TB treatment (for any duration of stay), in view of your treatment guidelines. For example, if your policy or general practice is to admit all TB patients for 2 months, the figure will be 100%.	hospd_prct_dstb	hospd_prct_mdr
18.1.3 - Estimated average duration of stay if hospitalized (days) If the actual duration of stay is available from the basic management unit register, please report. If not, please estimate the number of days a patient would spend in hospital "on average".	hospd_dur_dstb	hospd_dur_mdr

18.2 - Facility type

18.2.1 - If MDR-TB patients are hospitalized, in which type of facility are they most often treated) Primary-level hospital (or 'district hospital' or 'first-level referral'): has few specialities, mainly internal medicine, obstetrics-gynecology, pediatrics, and general surgery, or only general practice; limited general laboratory services; 30-200 beds. Secondary-level hospital (or 'provincial hospital'): highly differentiated by function with five to ten clinical specialities; 200-800 beds. Tertiary-level hospital (or 'central' or 'regional' hospital): highly specialized staff and technical equipment, e.g., cardiology, ICU and specialized imaging units; clinical services are highly differentiated by function; may have teaching activities; 300-1500 beds. Source: WHO guide to cost effectiveness analysis p215	hosp_type_mdr 140 Primary-level hospital 141 Secondary-level hospital 142 Tertiary-level hospital 2 Not applicable

19 - Implementation of WHO's Multisectoral Accountability Framework on ending TB (MAF-TB)

19.1 - MAF-TB assessment

Please outline steps taken for the implementation of MAF-TB at the national and sub-national level as described in Adaptation and implementation of the WHO Multisectoral Accountability Framework to end TB: Operational guidance.

19.1.1 - Have you conducted a MAF-TB assessment to inform planning and implementation?	maf_assessment 1 Yes 0 No

19.2 - Review mechanism, civil society and community engagement

19.2.1 - Is there a national multi-sectoral and multi-stakeholder accountability and review mechanism, under high-level leadership, to monitor and review progress towards ending TB? See https://www.who.int/publications/i/item/WHO-CDS-TB-2019.10	ms_review 1 Yes 0 No
(If 19.2.1 = yes) 19.2.2 - Do representatives of civil society and affected communities participate in the multi-sectoral accountability and review mechanism and in MAF-TB implementation?	ms_review_civil_soc 1 Yes 0 No

19.3 - Inter-ministerial collaboration

Please indicate how the following ministries or their equivalents are engaged in the national TB response

19.3.1 - Agriculture	min_agg_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)
19.3.2 - Defence	min_def_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)
19.3.3 - Education	min_edu_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)
19.3.4 - Finance	min_fin_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)

19.3.5 - Justice	min_jus_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)
19.3.6 - Labour	min_lab_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)
19.3.7 - Social development	min_dev_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)
19.3.8 - Transport	min_tra_collab 230 Advocacy, information, education, and communication 231 TB prevention and care 232 Patient support including economic, social or nutritional benefits 6 Not engaged 7 Not applicable (because there is no such ministry or its equivalent)
19.3.9 - Other (please describe briefly the ministry/sector and the area of collaboration or leave empty if not applicable)	other_min_collab

19.4 - Linkages with the private sector

19.4.1 - Have you established links with the private sector as part of the MAF-TB implementation process?	private_sector_link 1 Yes 0 No

19.5 - Reporting and reviewing

19.5.1 - Does the National TB Programme (or equivalent) produce a publicly available annual report about the status of the TB epidemic and progress in response efforts?	annual_report_published 1 Yes 0 No
(If 19.5.1 = yes) 19.5.2 - Please provide the web link to latest annual report if available	annual_report_url

19.6 - MAF-TB implementation plan

19.6.1 - Have you developed a MAF-TB component in the national strategic plan or a standalone MAF-TB implementation plan?	maf_implementation_plan 1 Yes 0 No

19.7 - Health and social benefits for people with TB

19.7.1 - Is there a national policy to specifically provide social protection services to TB-affected individuals? Social protection services are designed to prevent and reduce poverty, vulnerability and social exclusion. See Guidance on social protection for people affected by tuberculosis	social_protn 1 Yes 0 No 3 Don't know
19.7.2 - Free access to TB diagnostic testing for people with signs or symptoms presumptive of TB TB diagnostic testing includes all WHO-recommended rapid diagnostics, chest x-rays, culture and smear microscopy	free_access_tbdx 1 Yes 0 No 3 Don't know
19.7.3 - Free access to TB treatment and related medical services	free_access_tbtx 1 Yes 0 No 3 Don't know
19.7.4 - Enablers to adhere to TB treatment	enable_tx_adherence 1 Yes 0 No 3 Don't know
(If 19.7.4 = yes) 19.7.5 - Does funding for enablers to adhere to TB treatment include funding from the government?	gov_funding_tx_enablers 1 Yes 0 No 3 Don't know
19.7.6 - Conditional and/or unconditional cash-transfers	cash_trans 1 Yes 0 No 3 Don't know
(If 19.7.6 = yes) 19.7.7 - Does funding for conditional and/or unconditional cash-transfers include funding from the government?	gov_funding_cash_trans 1 Yes 0 No 3 Don't know
19.7.8 - Measures to ensure food security	food_security 1 Yes 0 No 3 Don't know
19.7.9 - Measures to compensate for loss of income	income_loss 1 Yes 0 No 3 Don't know
19.7.10 - Other (please describe briefly or leave empty if not applicable)	other_social_protn

B – Provisional monthly or quarterly data collection

Only shown to a subset of countries (criteria: use of dynamic models to estimate incidence or in high TB burden list or in agreement with regional offices). The country list was inherited from the 2025 data collection round but restricted to countries that had reported some provisional data during 2025. Those that provided quarterly data in 2025 are now shown the quarterly reporting form for 2026. Those that provided monthly data in 2025 are now shown the monthly reporting form for 2026.

Monthly Data

Please provide the provisional number of people with a new episode (new, recurrent, or unknown previous treatment history) of TB (all forms, including people with drug-resistant TB) that were notified each month as soon as the month has ended. WHO will use this ongoing reporting for early monitoring of trends and timely reaction to unusual changes. This approach is consistent with new guidance on TB surveillance that recommends monthly monitoring of TB case notifications.

	2025-09	2025-10	2025-11	2025-12	2026-01
Provisional number of people with a new episode (new, recurrent, or unknown previous treatment history) of TB (all forms, including people with drug-resistant TB) notified.	newinc_prov	newinc_prov	newinc_prov	newinc_prov	newinc_prov

Quarterly Data

Please provide the provisional number of people with a new episode (new, recurrent, or unknown previous treatment history) of TB (all forms, including people with drug-resistant TB) that were notified each quarter as soon as the quarter has ended. WHO will use this ongoing reporting for early monitoring of trends and timely reaction to unusual changes. This approach is consistent with new guidance on TB surveillance that recommends monthly monitoring of TB case notifications.

	2025-Q1	2025-Q2	2025-Q3	2025-Q4	2026-Q1
Provisional number of people with a new episode (new, recurrent, or unknown previous treatment history) of TB (all forms, including people with drug-resistant TB) notified.	newinc_prov	newinc_prov	newinc_prov	newinc_prov	newinc_prov